WEEKLY EPIDEMIOLOGICAL REPORT

A publication of the Epidemiology Unit
Ministry of Health & Mass Media

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WER SRI LANKA 2026

Vol. 53 No. 37 07th – 13th Sep 2026

Plague: An Old Disease with a Persistent Presence - II

This is the second article of two in a series on "Plague: An Old Disease with a Persistent Presence "

Types of plague

There are three main clinical forms of plague infection including bubonic, pneumonic and septicaemic (WHO, 2026) .

- **Bubonic** plague is the most common form of plague and is caused by the bite of an infected flea. Plague bacillus enters and travels through the lymphatic system to the nearest lymph node where it replicates itself lead to inflamed, tense and painful lymph node which called a 'bubo'. At advanced stages, the inflamed lymph nodes turn into open sores filled with pus. Human-to-human transmission is rare but in advanced stages, it can spread to the lungs, causing pneumonic plague, or to the bloodstream, causing septicaemic plague (WHO, 2026).
- **Pneumonic** plague, or lung-based plague, is the most virulent. The incubation period is very short as 24 hours. This can be transmited via respiratory particles to other humans. Although untreated pneumonic plague is fatal, recovery rates are high if detected and treated in time (within 24 hours of onset of symptoms (WHO, 2026).
- **Septicaemic** plague occurs when *Y. pestis* multiplies rapidly in the bloodstream, which lead to severe complications, including organ failure, tissue damage, respiratory distress and bleeding under the skin. Septicemic plague is fatal if left untreated (WHO, 2026).

The main routes of human infection

1. Flea bites - the commonest route, causing bubonic plague. Risk rises when rodents die in large numbers and their fleas move to humans.
2. Direct contact with infected animals - skinning, butchering or handling sick or dead rodents, marmots, hares, camels or pets, through breaks in the skin.
3. Eating undercooked infected meat - repeatedly documented in Mongolia and Inner Mongolia after marmot consumption.
4. Respiratory droplets - from a patient with pneumonic plague (or an infected cat) to close contacts, usually within about 2 meters.
5. Laboratory exposure - inhalation of aerosols from cultures or specimens, which is why *Y. pestis* is handled under biosafety level 3 containment.

Prevention and control

Plague cannot be eradicated from its wild reservoirs, so control aims to prevent human exposure, detect cases early and stop person-to-person spread (WHO,2026).

Community and individual protection

- Avoid hunting, skinning or eating marmots and other wild rodents, and never handle sick or dead animals with bare hands.
- Use insect repellent and protective clothing in endemic rural areas; treat pets for fleas.
- Keep homes, food stores and markets rodent -proof: store food and waste securely and remove harborage.

1. Strengthening Yellow Fever Preparedness in Sri Lanka: A Public Health Perspective 1
2. Summary of distribution of notified diseases reported by MOH (31st – 06th Sep 2026) 3
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Vector and rodent control

- In Sri Lanka, plague prevention should include monitoring flea populations and implementing appropriate insecticide-based flea control in rodent-infested areas, particularly before rodent control measures, to minimise the risk of infected fleas transmitting Yersinia pestis to humans.
- plague prevention should include regular monitoring of rodent populations, reporting unusual rodent die-offs, and implementing appropriate rodent control measures alongside environmental sanitation and rodent-proofing to reduce human exposure to potentially infected rodents and fleas.

Healthcare measures

- Early recognition: Suspect plague in patients presenting with sudden fever, painful swollen lymph nodes (buboes), or acute respiratory symptoms, particularly when there is a relevant epidemiological exposure.
- Prompt treatment: Initiate appropriate antimicrobial treatment immediately when plague is suspected. Treatment must not be delayed while awaiting laboratory confirmation. Provide supportive care according to the patient's clinical condition.
- Infection prevention and control: Apply standard precautions and appropriate additional precautions. Patients with suspected pneumonic plague should be isolated, with droplet precautions implemented according to national guidance. Healthcare workers should use appropriate PPE and follow hand hygiene and environmental cleaning procedures.
- Notification and contact management: Promptly notify public health authorities, investigate potential exposures, identify and assess close contacts, and arrange monitoring and post-exposure prophylaxis where indicated.

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Table 1: Distribution of Notified Diseases reported by Medical Officers of Health 31st Aug – 06th Sep 2026 (36th Week)

RDHS	Dengue Fever		Dysentery		Encephalitis		En. Fever		F. Poison-		Leptospirosis		Typhus		Viral Hep.		H. Rabies		Chickenpox		Meningitis		Leishman.		Tuberculosis		Leprosy		WRCD	
	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	A	B	T*	C**
Colombo	186	19316	0	18	0	2	0	4	0	21	3	236	0	2	2	17	0	0	10	409	3	83	0	4	48	1317	0	117	**28**	**97**
Gampaha	86	16048	0	33	0	34	0	0	1	25	6	372	0	6	1	10	0	0	3	598	7	222	0	39	20	731	0	56	**9**	**74**
Kalutara	78	7742	0	35	1	4	0	11	1	63	12	315	0	9	0	18	0	0	12	650	0	103	0	13	11	401	8	73	**14**	**90**
Kandy	8	6559	0	40	0	9	0	7	0	38	0	146	0	27	0	19	0	0	2	495	0	101	0	43	8	361	0	19	**0**	**100**
Matale	4	2212	1	20	0	5	0	2	0	7	2	156	0	3	0	8	0	0	6	160	1	66	5	381	4	134	0	25	**5**	**93**
Nuwara Eliya	4	1062	0	37	0	5	0	3	0	16	1	218	0	40	0	14	0	0	5	358	1	152	0	2	3	185	0	9	**13**	**100**
Galle	38	6133	0	30	0	13	0	8	0	60	3	416	0	33	2	25	0	0	6	771	1	135	0	2	12	309	2	48	**35**	**89**
Hambantota	21	2786	2	47	0	2	0	0	3	46	3	204	2	28	1	13	0	0	2	173	0	54	8	390	4	104	0	22	**57**	**100**
Matara	20	6446	1	22	0	4	0	2	0	11	9	296	1	18	0	17	0	0	5	404	2	141	1	108	2	108	1	26	**0**	**100**
Jaffna	17	1067	6	76	0	6	1	20	1	96	0	55	0	243	24	25	0	0	4	288	2	44	0	1	4	130	0	12	**92**	**100**
Kilinochchi	0	647	0	12	0	1	0	8	0	1	0	76	0	12	0	3	0	1	1	97	0	14	0	2	0	26	0	2	**0**	**100**
Mannar	1	537	0	1	0	3	0	0	0	3	1	47	0	4	0	3	0	0	1	65	0	21	0	5	2	39	0	5	**17**	**100**
Vavuniya	1	338	1	22	0	4	0	2	0	8	0	102	0	4	0	1	0	0	0	123	0	34	0	23	1	52	0	7	**0**	**83**
Mullaitivu	1	105	0	6	0	2	0	1	0	13	0	35	0	6	0	2	0	0	0	13	0	10	0	10	1	24	0	7	**0**	**100**
Batticaloa	14	1564	1	67	0	14	0	2	0	21	0	146	0	2	0	16	0	0	7	257	3	77	0	22	2	114	1	84	**0**	**100**
Ampara	19	674	0	53	1	2	0	1	0	16	1	129	0	2	1	17	0	0	3	278	1	64	0	17	2	42	0	21	**7**	**100**
Trincomalee	8	759	1	25	0	6	0	2	0	15	0	79	0	8	0	4	0	0	0	188	0	31	0	18	6	108	0	7	**98**	**100**
Kurunegala	29	3429	1	23	0	18	0	3	0	66	2	317	1	31	0	13	0	1	2	610	1	297	0	329	7	228	0	51	**11**	**98**
Puttalam	12	1881	1	19	0	11	0	0	0	9	0	197	0	26	0	7	0	4	5	131	2	141	0	15	6	131	1	24	**14**	**93**
Anuradhapura	21	1132	0	19	0	9	0	3	2	53	3	205	0	26	0	13	0	0	11	448	1	125	4	576	3	189	0	50	**3**	**100**
Polonnaruwa	8	832	0	25	0	7	0	2	0	59	0	225	0	7	0	25	0	1	2	383	0	57	3	396	3	63	0	67	**0**	**90**
Badulla	27	1727	1	34	0	8	0	3	0	13	4	208	2	44	2	120	0	0	4	339	1	206	4	97	7	182	0	15	**3**	**100**
Monaragala	6	1275	0	15	0	3	0	2	0	10	1	226	1	31	4	56	0	1	2	191	0	86	3	173	1	94	0	19	**37**	**80**
Ratnapura	44	5329	1	31	1	8	0	7	1	20	18	708	0	31	1	15	0	0	0	290	3	89	2	161	7	293	1	31	**6**	**99**
Kegalle	19	2783	0	46	0	6	0	5	0	28	6	322	0	11	1	13	0	0	6	567	1	79	0	13	6	229	0	9	**14**	**93**
Kalmunai	8	1037	0	44	0	3	0	1	0	22	0	63	0	1	0	3	0	0	2	382	0	36	0	0	3	87	0	26	**6**	**100**
SRILANKA	680	93420	**17**	**800**	**3**	**189**	**1**	**99**	**9**	**740**	**75**	**5499**	**7**	**655**	**39**	**477**	**0**	**8**	**101**	**8668**	**30**	**2468**	**30**	**2840**	**173**	**5681**	**14**	**832**	**18**	**95**

Source: WRCD module of the EPINET.
A = Cases reported during the current week;
T*=Timeliness refers to returns received on or before 06th Sep, 2026.
B = Cumulative cases for the year.
Total number of reporting units 363
C**-Completeness;
C**-Completeness;

Table 2: Selected Vaccine Preventable Diseases & AFP 31st Aug– 06th Sep 2026 (36th Week)

Disease	No. of Cases by Province									Number of cases during current week in 2026	Number of cases during same week in 2025	Total number of cases to date in 2026	Total number of cases to date in 2025	Difference between the number of cases to date in 2026 & 2025
	W	C	S	N	E	NW	NC	U	Sab					
AFP[1]	01	00	00	00	00	00	00	01	00	02	03	44	44	0%
Diphtheria	00	00	00	00	00	00	00	00	00	00	00	00	00	0 %
Mumps[2]	00	00	00	00	00	00	00	00	00	01	02	104	106	-1.8%
Measles[3]	00	00	00	00	00	02	01	00	00	03	00	60	02	2900 %
Rubella[3]	00	00	00	00	00	00	00	00	00	00	00	00	00	0 %
CRS[2]	00	00	00	00	00	00	00	00	00	00	00	00	00	0 %
Tetanus[2]	00	00	00	00	00	00	00	00	00	00	01	04	04	0 %
Neonatal Tetanus[2]	00	00	00	00	00	00	00	00	00	00	00	00	00	0 %
Japanese Encephalitis[3]	00	00	00	00	00	00	00	00	00	00	01	00	04	-100 %
Whooping Cough[2]	00	00	00	00	00	00	00	00	00	00	01	21	16	31.2 %

Key to Table 2

Provinces: **W**: Western, **C**: Central, **S**: Southern, **N**: North, **E**: East, **NC**: North Central, **NW**: North Western, **U**: Uva, **Sab**: Sabaragamuwa.

Data Sources:
Weekly Return of Communicable Diseases: Diphtheria, Mumps, Tetanus, Neonatal Tetanus, Whooping Cough.
Special Surveillance: AFP, Measles, Rubella, CRS.

AFP[1] = No Polio cases
Mumps[2], CRS[2], Tetanus[2], Neonatal Tetanus[2], Whooping Cough[2]—Clinically and/ or laboratory confirmed cases
Measles[3], Rubella[3], Japanese Encephalitis[3]— Laboratory Confirmed cases

AFP—Acute Flaccid Paralysis
CRS = Congenital Rubella Syndrome
NA = Not Available

AFP and all Vaccine Preventable Diseases should be investigated by the MOH Personally.

All patients presenting with fever and a maculopapular rash must be promptly notified and investigated. Blood samples and throat swabs should be collected at the appropriate time window

Comments and contributions for publication in the WER Sri Lanka are welcome. However, the editor reserves the right to accept or reject items for publication. All correspondence should be mailed to The Editor, WER Sri Lanka, Epidemiology Unit, P.O. Box 1567, Colombo or sent by E-mail to **chepid@sltnet.lk.** The Epidemiology Unit should be formally acknowledged in all resulting publications as the primary data source.

How to cite: Epidemiology Unit, Ministry of Health, Sri Lanka. (Year). Weekly Epidemiological Report, Vol. [Volume Number], No. [Issue Number, Page numbers].

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